

## FOOTBALL PITCH-SIDE SUDDEN CARDIAC ARREST MANAGEMENT

### Team Member Roles & Equipment Assignments

First-response FoP medical team = Red + Blue + White + Green + Orange. Second-response FoP medical team = Yellow (First Aiders). Match Doctor = Black.

**Black** (Match Doctor): Leader of the Field of Play (FoP) medical team. Hands-off on the left flank. Directs timing, commands movement, alerts the ambulance, instructs **Green** on drugs, and calls the lift – only after the 3rd shock (earlier if ROSC).

**Orange** (Airway Specialist): Directs airway management, inserts the Laryngeal Mask Airway (LMA) (or OPA), manages Bag-Valve Mask (BVM) ventilations, and controls the log-roll from the head.

**Red** (Compressor 1): Assesses responsiveness, checks signs of life, performs Cycle 1 of CPR, and takes the chest in the log-roll.

**Blue** (Compressor 2): Brings the O<sub>2</sub> tank onto the field, connects it to the BVM reservoir (15 L/min), takes the pelvis in the log-roll, and takes over compressions from **Red** at 2 minutes (Cycle 2).

**White** (Board Handler): Brings the spinal board (± basket), spider straps and splints. Places the board below the feet, in line with the player, 2–3 lengths away. Takes the legs in the log-roll and slides the board in with the **Yellow First Aiders**.

**Green** (AED Operator / Meds): Brings the emergency care bag and AED onto the field, operates the AED, checks all clear before every shock, tracks the 10-second movement windows, and prepares drugs / inserts the IV cannula if competent.

**Yellow First Aiders** (second-response team, 4 rescuers): Stand at the feet beside the spinal board, facing the player. Dedicated crew for lifting and carrying the spinal board – only on **Black's** command.

### Step 1: Initial Response & Assessment

The Scenario: A player suffers a non-contact collapse on the football pitch.

The Approach: The Field of Play (FoP) medical team enters the pitch immediately – do not wait for the referee – and takes organized tactical positions around the body.

The First Check: **Red** taps the player and assesses responsiveness. **Red** states clearly: "Player is unresponsive!"

The Signs of Life Check: **Red** checks for breathing and a carotid pulse simultaneously for no more than 10 seconds. **Red** confirms: "No breathing, no pulse. No signs of life!" Gasping or slow seizure-like movement = cardiac arrest, not a fit.

The Radio Call: **Black** immediately uses the radio/comms to alert stadium medical control: "Sudden cardiac arrest protocol activated. Ambulance on standby for immediate field entry."

### Step 2: Immediate CPR & Advanced Airway Setup

The Compressions: **Red** immediately commences high-quality chest compressions (5–6 cm, 100–120/min, full recoil): 30 compressions to 2 breaths until the LMA is in, then continuous compressions.

The AED Application: **Green** arrives with the emergency care bag and the AED, exposes the chest and applies the pads at once – do not wait for the current 30-count (upper right chest and lower left armpit). **Red** does not stop compressions while the pads are being applied.

The Airway: Once compressions and the AED are running, **Orange** moves to the head, opens the airway with a jaw thrust and inserts an LMA (if unavailable, an Oropharyngeal Airway [OPA]).

The Oxygen Line: **Blue** arrives with the O<sub>2</sub> tank, opens the flow to 15 L/min and attaches the line to the bag/valve reservoir.

Ventilation Delivery: **Orange** attaches the bag with the reservoir to the LMA and gives one breath every 10 compressions (face mask only, or LMA leaking: 30:2).

Vascular Access & Drugs (if time and circumstances permit): **Green** opens the emergency care bag beside the patient. A competent rescuer (as designated by **Black**) inserts the IV cannula, while **Green** prepares the resuscitation drugs to be given on **Black's** order.

### Step 3: AED Analysis

The Trigger: The AED announces: "Stop CPR. Analyzing rhythm. Do not touch the patient."

The Action: **Red** instantly stops compressions and hovers hands above the chest. **Orange** pauses ventilations.

### Step 4: First Shock

The Trigger: The AED announces: "Shock advised. Charging. Stand clear."

The Oxygen: LMA connected (closed circuit) – leave the BVM and oxygen connected; no need to turn

the O<sub>2</sub> off. Face mask only (no LMA) – **Orange** lifts the BVM off the face for the shock.

The Action: **Green** checks the full length of the patient's body and yells: "I'm clear, you're clear, everyone's clear!"

The Delivery: **Green** presses the flashing shock button. The shock is delivered.

### Step 5: Board Transfer (after the first shock)

The Command: **Black**: "Orange, you have the roll." **Orange** holds the head and commands: "Roll on three. One, two, three, ROLL."

The Log-Roll: **Red** takes the chest, **Blue** the pelvis and **White** the legs, rolling the patient onto their side.

The Slide: **White** and the **Yellow First Aiders** slide the spinal board underneath. Strap: player + spinal board + AED = one unit ( $\pm$  basket for the carry). **White**: "Board is in."

### Step 6: CPR until the next AED analysis

The Compressions: CPR continues on the board until the AED prompts the next analysis. **Blue** takes over compressions from **Red** at 2 minutes.

### Step 7: Second Shock – stay on the pitch

The Action: If a shock is advised, **Green** checks all clear and delivers the shock (LMA connected: O<sub>2</sub> stays on).

No carry yet: the team stays on the pitch and CPR continues for 2 minutes.

### Step 8: Third Shock + The 10-Second Dash (10 Seconds)

The Rule: Extricate only after the 3rd shock. If ROSC comes before that, extricate straight away – keep the pads on and monitor.

The Command: The moment the 3rd shock finishes, **Black** commands: "Ready to lift. On three. One, two, three, LIFT."

The Carry: The **Yellow First Aiders** reply "Lifting on your command, Black," lift the spinal board and move swiftly toward the ambulance tunnel – keep pace, stay silent.

The Medical Escort: **Orange** walks at the head, keeping the LMA and BVM secure. **Blue** carries the O<sub>2</sub> tank, keeping it connected to the bag. **Green** carries the emergency care bag and the AED.

**Black**, **Red** and **White** manage the pace and clear the path.

The Timekeeper: **Green** counts the seconds out loud: "1... 2... 3... 4... 5... 6... 7... 8... 9... 10!"

### Step 9: The Drop, Swap, CPR until AED analysis (1 Min, 50 Secs)

The Trigger: **Green** reaches the count of "10".

The Command: **Black** commands: "STOP. Board DOWN. Start CPR."

The Drop: The **Yellow First Aiders** place the board flat on the ground and step back.

The Swap & CPR: Fresh-handed **Red** or **Blue** instantly kneels on the board and gives deep, high-quality chest compressions.

Oxygenated Airway Care: **Orange** stabilizes the LMA and delivers high-flow oxygenated ventilations via the BVM.

The Rest of the Cycle: The team stays in this position for the remaining 1 minute and 50 seconds (at least 2 minutes of CPR) until the AED prompts the next analysis.

### Step 10: Repeat

Repeat: analysis → shock if advised → 10-second dash → board down → CPR until the player is inside the ambulance. Prefer the ambulance onto the field.